up rapidly. In turn, factories will have to escalate the wages they pay to managers, engineers, and supervisors. Now factories that produce for the domestic market and don't face competition can pass their higher costs on. But factories that export cannot, so they will cut down on operations and even start shutting down. This is one example of a phenomenon called Dutch Disease, which makes aid recipients less competitive. Subramanian and I show that in countries that received more aid in the 1980s and 1990s, the export-oriented, labour-intensive industries not only grew more slowly than other industries – suggesting that aid did in fact create Dutch Disease – but the manufacturing sector as a whole also grew more slowly. Again it's sobering to think that by constraining the growth of manufacturing, aid inflows may have prevented poor countries from taking the path to growth followed first by the East Asian tigers and now by China.

That said, Dutch Disease is not a terminal condition. It can be mitigated through sensible policies. But to do so, one must first acknowledge its existence and its pernicious effects. Similarly for other possible diseases caused by aid.

THERE IS HOPE

To ignore the past, or to read only rosy lessons from it, is to condemn oneself to relive it. While it would be churlish to deny that many poor countries have made tremendous progress in creating the conditions for sustained growth, it doesn't serve the citizens of poor countries either if we say that all the problems of the past are well and truly in the past. While no one has the 'magic bullet' for growth, there are some things that do seem important. These include sensible macroeconomic management, with fiscal discipline, moderate inflation, and a reasonably competitive exchange rate; laws and policies that create an environment conducive to private sector activity with low transaction costs; and an economy open for international